

Name	: Saroj Agarwal	Patient ID	: 0010663186
Date of Birth	: 27-08-1962	Age/Sex	: 63 Years/F
Performing Date	: 21-02-2026 02:55 PM	Order No.	: 0040725914
Referred By	: Shah Darshit	Report Date	: 23-02-2026 06:06 PM
Study Description	: WB FDG PET - CECT scan		

18F-FDG WB PET-CT Scan

Clinical History: 63-year-old female, presented with complaints of breathlessness, productive cough, bilateral pedal edema for the past 2 months; persisting hematuria since 12/02/26. Chest X Ray showed left sided pleural effusion. ICD was inserted (12/02/26), pleural fluid cytology revealed clusters of atypical cells suggestive of carcinoma with large cell morphology. CT Chest + Abdomen (13/02/26): Bilateral pleural effusion with left sided enhancing pleural thickening, hyperenhancing nodule in segment VIII of liver. She received C #1 Pacliquelip + Carboplatin (20/02/26- Day 1). Patient has a history of Carcinoma endometrium, operated 3 years ago. Scan for disease status assessment.
Labs (20/02/26): TLC - 13.88 (Neutrophil - 89.3%); INR - 2.4
Urine Routine (20/02/26): Pus cells - >100/hpf; RBC - >100/hpf; Bacteria - Present

Technique: 7.28mCi of 18F-FDG was injected intravenously and PET-CT scan was acquired from vertex to mid-thigh after one hour. Contrast-enhanced CT scan was performed as part of the PET-CT protocol. Breath-hold CT of the lungs was acquired as well. Standardized uptake values (SUV) were calculated based on body weight.

FINDINGS:

Excessive motion artefacts seen- suboptimal assessment.

Brain:

Normal distribution of metabolic activity is noted intracranially. No focal area of abnormal increased tracer avid definite lesion seen. Tiny gliotic foci seen in right ganglio-capsular region - sequelae to ischemic insult.

Head & Neck:

Nasopharynx and hypopharynx are unremarkable.

Localized linearly increased FDG uptake is seen along the minor salivary glands in right cheek - SUVmax 7.7 - ? inflammatory.

Intense FDG uptake noted along the inner surface of buccal mucosa- right greater than left - SUVmax 13.85: Likely excessive physiological stimulation/inflammatory.

Thyroid appears normal in size and shows no abnormal tracer uptake.

Moderate to high grade FDG avid left cervical level II, III, IV and V partly necrotic nodes are seen. Largest - left VC node measures 2 x 2.4 cm, SUVmax 12.74

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Thorax:

Lungs -

- Bilateral moderate pleural effusion seen with secondary lower lobe subsegmental collapse. Right sided pleural effusion shows high grade increased FDG uptake - SUVmax 6.15; mild to moderately heterogeneously increased FDG uptake seen in left pleural lining - SUVmax 5.6 - with no significantly increased tracer uptake in the fluid.
- Left ICD tube noted in situ with its tip reaching the anterior subpleural region with secondary mild pneumothorax.
- Dependant infiltrates and areas of oedematous interstitial thickening seen at places. Within the limitation of suboptimal breathhold- no obvious FDG avid lung parenchymal soft tissue lesion seen.

Low-to-moderate grade FDG avid few mediastinal - prevascular, subcarinal nodes are seen. Representative - 1 cm, SUVmax 4.12 (subcarinal)

Heart and other mediastinal structures are unremarkable.

Moderate to high heterogeneously increased FDG uptake is seen in oedematous soft tissue thickening in left axilla along the lateral chest wall at the site of insertion of ICD tube - SUVmax 9.29: Likely postprocedural inflammatory changes
Secondary surgical emphysema noted

Moderate to high increased FDG uptake is seen in multiple enlarged left axillary level I, II & right infraclavicular, retropectoral nodes -representative - largest left axillary node measures ~ 1 x 1.3 cm, SUVmax 7.45

No focal FDG-avid abnormality seen in breasts

Abdomen & pelvis:

Post hysterectomy status.

No evidence of focal abnormal tracer avid soft tissue lesion seen at the excision site.

Mild ascites seen along the perihepatic, perisplenic region and pelvic cavity.

Low-grade FDG uptake noted in few of the sites of pelvic fluid collection. Representative - SUVmax 3.54

Multiple areas of fibrofatty stranding and nodularity seen in supra mesocolic omentum predominantly - SUVmax 4.17

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FDG avid following nodes are seen:

- Abdominal - pancreaticoduodenal axis, periportal, portacaval. Largest ~ 1 x 1.4 cm, SUVmax 4.30
- Bilateral inguinofemoral. Largest - left measures 2.3 x 1.4 cm, SUVmax 6.43

Non-FDG avid thin walled observations are seen in segment VIII and VI of liver - 2.7 x 3.6 cm (Segment VI)- ? cysts, ? old healed lesions.

Spleen, gallbladder, pancreas, adrenals and kidneys are unremarkable.

No abnormal focal increased tracer uptake seen in GI tract above the physiological activity.

Musculoskeletal:

Increased FDG uptake seen in intercostal muscles, diaphragm and diaphragmatic crura: likely excessive stimulation.

No focal FDG-avid lytic or sclerotic bone lesions seen.

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REMARKS:

(No previous PET-CT is available for comparison)

- Post hysterectomy status.
- Mild ascites seen along the perihepatic, perisplenic region and pelvic cavity with few areas of collection showing low-grade metabolic activity: Suspicious metastatic. Multiple areas of fibrofatty stranding and nodularity is seen in supra mesocolic omentum predominantly - metastatic.
- Moderate to high grade metabolically active abdominal and left cervical adenopathy - likely metastatic.
Moderate to high grade metabolically active inguinofemoral, left axillary and right infraclavicular/retropectoral adenopathy - likely metastatic involvement.
- Bilateral moderate pleural effusion seen with right effusion showing high grade increased metabolic activity: Metastatic.
Secondary lower lobe subsegmental collapse seen.

Left ICD noted in situ with misplaced tip reaching the anterior subpleural region with secondary mild pneumothorax. Mild to moderate heterogeneously increased metabolic activity seen in left pleura - no significantly increased metabolic activity in left pleural effusion.

- No evidence of abnormal increased metabolic activity seen anywhere else in the body

Histopathological correlation from left supraclavicular node can be considered.

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